St Alder Infirmary
Department of Dermatology
Leeds, United Kingdom

Date: 10 March 2026

To: Drug Safety Department, Aldermoor Pharmaceuticals Ltd
Re: Suspected adverse drug reaction

Dear Colleague,

I am writing to report a suspected adverse reaction in one of my patients, Mrs M.K., a 54-year-old woman with no significant past medical history.

Mrs M.K. was prescribed amoxicillin 500 mg capsules, one capsule orally three times daily, for a lower respiratory tract infection. She took the first dose on 2 March 2026.

On 4 March 2026 she developed a widespread pruritic maculopapular rash on the trunk and limbs, followed within hours by swelling of the lips and periorbital area. She attended the emergency department and was admitted for observation. Amoxicillin was stopped on admission. She was treated with intravenous chlorphenamine and oral prednisolone.

The rash and the facial swelling resolved completely by 7 March 2026 and she was discharged home. She was not taking any other medication at the time of the reaction.

In my opinion the reaction is probably related to amoxicillin.

Yours sincerely,

Dr Helen Carrow
Consultant Dermatologist
GMC 7041188